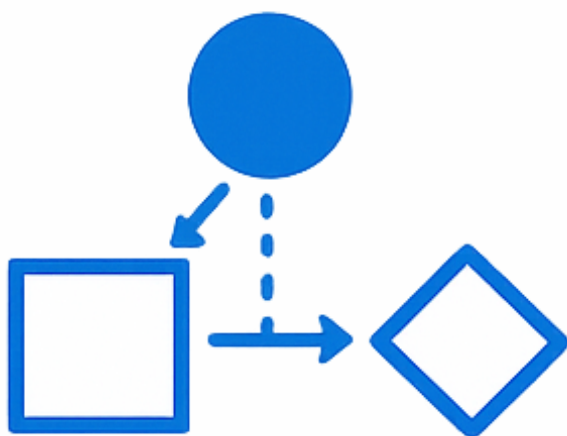




Assignment 2

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Qualitative Methods and Data Quality

Part I: Say-Do Gap Study and Part II: Radiology Data Quality Analysis

Date: March 2, 2026

Course: NDAB20010U Health Data and Interoperability

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AI Declaration

I declare that AI has only been used for grammatical correction.

AI has **not** been used to generate content, reasoning, or structure.

Contents

1	Part I: A Real-World Study of the Say-Do Gap in Modelling	4
1.1	Deliverable 1: Description of the Routine Based on the Interview	4
1.2	Deliverable 2: Event Log Representing the Observed Routine	5
1.3	Deliverable 3: Delta Sheet Summarising Differences Between Stated and Observed Practice	6
1.4	Deliverable 4: Reflections on the Modelling Process	7
2	Part II: Radiology Case Study — Data Quality, Conformance, and the Limits of Logs	9
2.1	Deliverable 5: Notebook Submission	9
2.2	Deliverable 6: Part II Reflection Questions	9
2.3	Deliverable 7: Notebook Reflection Questions	11
A	Supporting Tables for Part I	14
A.1	Core Deliverable Artefacts (Part I)	15
B	Supporting Tables and Code for Part II	19
B.1	Notebook Visualisations (Part II)	21
B.2	Custom DQ Assessment Method Code Excerpt	23
B.3	Textbook Figures Referenced in the Report	24
	References	29

1 Part I: A Real-World Study of the Say-Do Gap in Modelling

This part analyses one weekday morning routine across *work-as-imagined*, *work-as-done*, and *work-as-recorded*, with explicit links between claims and evidence.

1.1 Deliverable 1: Description of the Routine Based on the Interview

Task Requirement (Deliverable 1) *Based on the interview, describe the routine and identify activities, their order, and participants involved.*

Scope and notation This Part I study uses a single-case self-study design with one participant (the author). The chosen routine is a weekday morning routine.

The interview was conducted on February 18, 2026 as a self-recorded, semi-structured interview with open questions. Following Guide to Health Informatics (2026a), this stage is treated as *manual knowledge acquisition*: it captures *knowledge-as-stated* and establishes a first *work-as-imagined* model of the routine (Guide to Health Informatics 2026; Zajac 2026). This placement is consistent with textbook Figure 27.1 from *Guide to Health Informatics*, where model acquisition precedes formal computer reasoning (Appendix B.3, Figure 7).

Identifier convention used in this report:

- **Axx** = *activity* IDs from the interview-based model (e.g., A01-A09).
- **Exx** = *event* IDs from the observed event log (e.g., E03, E10).
- **Dxx** = *delta* IDs in the comparison sheet (e.g., D01-D06).
- **Mx** = *method* IDs from the Part II DQ toolkit (e.g., M2, M10).

The model notation *DCR* denotes *Dynamic Condition Response* graph modelling.

The routine boundary is defined as start when the alarm sounds and end when the apartment door closes, based directly on transcript lines L21-L22 in Table 2. The baseline sequence (wake-up, hygiene, dressing/packing, breakfast, checks, departure) is grounded in lines L02-L12, and variation points (snooze, breakfast substitution, key search, occasional roommate coordination) are grounded in lines L14-L18. This line-level grounding makes the interview model auditable.

Routine boundary:

- Start: alarm sounds.
- End: participant leaves home (door closes).

Table 3 presents the interview-based activity model, and Figure 1 shows the corresponding *interview-based DCR* graph exported from DCR Graph. The graph was built from activities A01-A09 and used as the baseline representation for comparison against the observed event log in Deliverables 2 and 3.

Assumptions used for modelling:

- The model represents a weekday baseline (not weekend behavior).
- The participant is the only consistent human actor in the routine.
- Expected times are recall-based interview estimates and are treated as approximate.

These assumptions delimit model scope and support transparent comparison between *knowledge-as-stated* and observed evidence in Deliverables 2 and 3.

Interview transcript evidence The full line-numbered transcript excerpt used as evidence for Deliverables 1-4 is provided in Appendix A, Table 2.

Interview-based activity model The full interview-based activity table and DCR figure are provided in Appendix A, Table 3 and Figure 1. These appended artefacts are the baseline *work-as-imagined* model used for Deliverables 2 and 3.

1.2 Deliverable 2: Event Log Representing the Observed Routine

Task Requirement (Deliverable 2) *Create an event log representing the observed routine as a .csv file with timestamps, event labels, and relevant context/notes.*

The observation was conducted on February 20, 2026 (06:45-07:30), two days after the interview on February 18, 2026. This separation supports comparison between interview elicitation and situated execution.

As described in Chapter 27 and the course methods slides, *observation* complements *interviews* by capturing situated judgement and practical adaptations that are often missing from recollection-based accounts (Guide to Health Informatics 2026; Zajac 2026). To make data collection explicit and reproducible, a short observation guide was used (Appendix A, Table 1). Notes were split into *descriptive notes* and *reflexive notes*, and only descriptive notes were transcribed into the event log (Zajac 2026).

The full machine-readable event log is submitted as a separate CSV artifact with timestamps, event labels, and context notes. This file is the *work-as-recorded* layer of the study and the empirical basis for the *observed DCR* comparison in Deliverables 3-4.

The submitted CSV uses the following fields: `case_id`, `event_id`, `timestamp` (ISO datetime), `event_label`, `actor`, `tool`, `context_note`, and `related_activity_id` (A01-A09 or NA).

Table 4 in Appendix A provides the observed event-log excerpt with activity references. Relative to the transcript-based baseline (Table 2, lines L02-L12 and L14-L18), the key comparison anchors are E03 (extra coordination), E09 (order shift), and E10 (workaround).

The observation confirms the same overall routine boundary as Deliverable 1, but adds practical variation and coordination not present in the interview baseline. In particular, one extra coordination event (E03), one order change (E09 relative to A04/A05), and one explicit workaround (E10) are central for the delta sheet in Deliverable 3. These events represent *work-as-done* under actual constraints.

1.3 Deliverable 3: Delta Sheet Summarising Differences Between Stated and Observed Practice

Task Requirement (Deliverable 3) *Compare Deliverable 1 and Deliverable 2 and produce a delta sheet categorised into missing events, extra events, different order, timing distortions, workarounds/coordination steps, and relabeling/ambiguity.*

The comparison maps interview activities (A01-A09) to observed events in Deliverable 2. A delta was registered when at least one of the following differed: event presence, order, timing, workaround behavior, or interpretation of activity labels. Methodologically, this follows Chapter 27's logic of comparing observed process traces with a pre-existing process description to identify frequent exceptions and model limits (Guide to Health Informatics 2026).

Table 5 in Appendix A summarises the full delta comparison between stated and observed routine. Key deltas are:

- *Missing event*: no distinct seated breakfast-and-schedule-review step in observation.
- *Extra event*: additional roommate coordination event appears only in observed data.
- *Different order*: bag packing occurs after breakfast preparation begins.
- *Timing distortion*: hygiene block starts later than stated baseline.
- *Workaround/coordination step*: missing lock triggers ad hoc borrowing via messaging.
- *Relabeling/ambiguity*: “breakfast” shifts from sit-down meal to banana plus coffee-to-go.

All six required categories are documented in Table 5: missing events, extra events, different order, timing distortions, workarounds/coordination steps, and relabeling/ambiguity.

Overall, the *interview-based DCR* (Deliverable 1) captures *knowledge-as-stated* and *work-as-imagined*, while the observed event sequence (*observed DCR* evidence) captures *knowledge-as-discovered*, *work-as-done*, and *work-as-recorded*. The delta sheet therefore provides traceable evidence of the *say-do gap*.

1.4 Deliverable 4: Reflections on the Modelling Process

Task Requirement (Deliverable 4) *Reflect on six modelling questions: data missed/misrepresented, activity definition differences, event-log constraints, automation potential, variations revealed by observation, and why qualitative discovery matters for process modelling.*

1. **Question 1:** *What kinds of data were missed or misrepresented in the interview-based model and why?* **Answer:** The interview model captured routine intent, but missed situated details that appeared in observation: micro-coordination (E03), recovery behavior (E10), and omission of the planned seated breakfast-and-schedule step. This pattern is expected in *manual knowledge acquisition: interviews* primarily capture recalled intent, while *observation* captures situated adaptation (Guide to Health Informatics 2026; Zajac 2026). In this case, the interview model therefore overestimated routine linearity.
2. **Question 2:** *What counts as an activity in each model, and how did this differ?* **Answer:** In Deliverable 1, the *interview-based DCR* uses intention-level activities (A01-A09). In Deliverable 2, the event log (the empirical basis for the *observed DCR*) uses timestamped observable actions. This granularity mismatch means one DCR activity can map to multiple logged events. The same routine can therefore appear stable at intention level but variable at event level.
3. **Question 3:** *How did the event log constrain what could be modelled?* **Answer:** The event log supports strong analysis of order, timing, and deviations (*work-as-recorded*), but it captures motives and judgement only indirectly. Without contextual notes, causal interpretation of why a deviation occurred remains weak. This is why descriptive/reflexive note separation was retained in the method and documented in Appendix A, Table 1.
4. **Question 4:** *Do you think that the event log could be captured automatically by a system? Why or why not?* **Answer:** Automatic capture is strong for digital traces (alarm, app checks, timestamps), but weak for tacit coordination, local workarounds, and searches for missing items. In Chapter 27 terms, automated traces should be combined with *qualitative discovery* methods when the task includes social and contextual adaptation (Guide to Health Informatics 2026; Zajac 2026).
5. **Question 5:** *How did the observation help uncover variations or exceptions that were not mentioned in the interview?* **Answer:** Observation revealed concrete exceptions: changed order (E07 before E09), omitted planned step (A06 not distinct), and workaround behavior (E10). Applied to this case, Chapter 27 implies that process evidence should not only test conformance but also explain recurring exceptions that may be locally rational (Guide to Health Informatics 2026).

6. **Question 6:** *Why is it relevant to consider qualitative methods of discovery (e.g., observations) when building process models?* **Answer:** For this routine, model quality improved by triangulating *knowledge-as-stated* (interview + transcript), *knowledge-as-discovered* (observation), and *work-as-recorded* (event log). The resulting model is more realistic than any single method alone, consistent with Chapter 27's iterative *knowledge acquisition* approach (Guide to Health Informatics 2026).

2 Part II: Radiology Case Study — Data Quality, Conformance, and the Limits of Logs

This part synthesises notebook evidence (Deliverable 5) and two separate reflection sections: Deliverable 6 (assignment-brief wording) and Deliverable 7 (notebook-template wording).

2.1 Deliverable 5: Notebook Submission

Task Requirement (Deliverable 5) *Submit the completed notebook `assignment.ipynb` with the Part II workflow and method execution.*

The completed notebook is submitted as a separate artifact (`assignment.ipynb`), as required by the assignment brief.

The notebook contains the full Part II workflow (Parts 0–7): initial exploration, method selection and execution, custom *DQ dimension* design, *five-facet* synthesis (*Data, Source, System, Task, Human*), and *invisible-work* analysis.

Deliverable 6 and Deliverable 7 both interpret these outputs as *work-as-recorded* evidence, with different wording alignment (assignment brief vs notebook template).

2.2 Deliverable 6: Part II Reflection Questions

1. Initial exploration. What kind of data is logged in the file? Assuming data quality is good, how can the data be used to answer the RQs?

Answer:

2. Method selection justification. Include your completed method selection table from Part 3. Additionally, answer, why did you NOT choose the other methods? What trade-offs did you consider? How did the three research questions (RQs) influence your method selection?

Answer:

3. Results interpretation. Provide detailed interpretation of results from each chosen method:
 - Include key results.
 - Describe and interpret the results. Did they work as expected? Did they reveal any surprising insights? Could they be improved?
 - Were your expectations correct?
 - Which facet(s) does this result primarily inform about?

Answer:

4. Your proposed DQ dimension. Define your dimension (from Part 5), include the code for your custom assessment method and explain:
- What specific pattern or issue in the data motivated this dimension? Reference your exploration code above. Provide concrete examples.
 - Which research question(s) would be affected by this quality issue? How?
 - Include your facet mapping table

Answer:

5. Per-Facet Data Quality Synthesis. For each of the five data quality facets, interpret the results you got when running selected methods and assess the quality of the dataset. You can use these questions as guidance:
- What did your methods reveal about data quality at a facet level?
 - What are the main data quality issues?
 - What could be the consequences in regard to the RQs?

Then discuss:

- Which facet was most challenging to assess with the available methods? Why?
- How else could these data quality facets be assessed?

Answer:

6. Invisible work and system design implications. Using your invisible work analysis (Part 7) and the workflow diagram (Figure 1), address the following:
- Include your completed table identifying at least three activities from the diagram that do not appear in the event log
 - For each activity, explain why it might not be logged (technical, social, economic reasons)
 - Reflect on how a process model built only from the event log would fail to capture these activities. What are the potential consequences for system design, in regard to each of the research questions?
 - How does your Part I experience (interview vs. observation) parallel the gap between event log and workflow diagram?

- What does the gap between logged data and actual practice tell us about building healthcare IT systems from “data-driven” approaches alone?
- Propose one concrete change to the logging system that would capture currently invisible work. Draw on concepts from Chapter 9 (Formal and informal information systems) to discuss trade-offs: Why is complete logging difficult or even unwelcome in real-world settings? How could negative impacts be mitigated?

Answer:

2.3 Deliverable 7: Notebook Reflection Questions

Important notes:

- You will write ~20-25% code (basic exploration, running analyses, creating visualisations)
- Focus is on **choosing appropriate methods**, **interpreting results**, and **critical analysis**
- **All numbered “Reflection Questions” should be answered in your main report (Deliverable 7), not in this notebook**
- There is no single “correct” set of methods — you must justify your choices
- In-notebook analyses are brief; deeper reflections go in your report

Part 3: Method Selection and Justification

Task : From the methods above (Methods 1-11), choose 4-5 methods to run.

Complete this table in your main report.

Method	DQ Dimension	Primary Facets	Relevant to RQ(s)	Why did you choose this?	Expected findings
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Example:

Method 1

Why did you NOT choose the other methods? What trade-offs did you consider? How did the three research questions (RQs) influence your method selection?

Answer:

→ **Reflection Question 1** (answer in your main report):

What kind of data is logged in the file? Assuming data quality is good, how can the data be used to answer the RQs?

Answer:

→ **Reflection Question 3** (answer in your main report):

Provide detailed interpretation of results from each chosen method:

- Include key results.
- Describe and interpret the results. Did they work as expected? Did they reveal any surprising insights? Could they be improved?
- Were your expectations correct?
- Which facet(s) does this result primarily inform about?

Answer:

→ **Reflection Question 4** (answer in your main report):

Expand on your proposed DQ dimension.

- Define your dimension.
- *What specific pattern or issue in the data motivated this dimension? Reference your exploration code above. Provide concrete examples.*
- *Which research question(s) would be affected by this quality issue? How?*
- Map the dimension relevancy in across the five facets

Facet Mapping:

Facet	Involvement (++, +, -)	Justification
Data		
Source		
System		
Task		
Human		

Answer:

→ **Reflection Question 5** (answer in your main report):

For each of the five data quality facets, interpret the results you got when running selected methods and assess the quality of the dataset. You can use these questions as guidance,

- What did your methods reveal about data quality at [a facet] level?
- What are the main data quality issues?
- What could be the consequences in regard to the RQs?

Then discuss:

- a) Which facet was most challenging to assess with the available methods? Why?
- b) How else could these data quality facets be assessed?

Answer:

7.1 Invisible Work Analysis

Complete the table with at least **3 activities** from the workflow diagram that do NOT appear in the log:

Activity from Diagram	Why not logged?	Which facet explains this?	Consequences	How could it be captured?
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→ **Reflection Question 6** (answer in your main report):

- a) For each activity, explain why it might not be logged (technical, social, economic reasons)
- b) Reflect on how a process model built *only* from the event log would fail to capture these activities. What are the potential consequences for system design, in regard to each of the research questions?
- c) How does your Part I experience (interview vs. observation) parallel the gap between event log and workflow diagram?
- d) What does the gap between logged data and actual practice tell us about building healthcare IT systems from “data-driven” approaches alone?
- e) Propose one concrete change to the logging system that would capture currently invisible work. Draw on concepts from Chapter 9 (Formal and informal information systems) to discuss trade-offs: Why is complete logging difficult or even unwelcome in real-world settings? How could negative impacts be mitigated?

Answer:

A Supporting Tables for Part I

This appendix contains supplementary Part I material referenced from Deliverables 1–4.

Table 1: Observation guide used during Deliverable 2

Focus	Observation prompt	Evidence target
Sequence and boundary	Which event marks start/end, and what is the observed order of actions?	Event IDs, timestamps, sequence notes
Tools and artefacts	Which tools/resources are used at each step?	Tool field, context notes
Interactions and coordination	Does any interaction with other people occur?	Actor field, coordination notes
Deviations from interview baseline	Which actions are skipped, added, reordered, or relabeled relative to A01-A09?	Related activity ID + delta notes
Adaptation and recovery	Which workaround or exception handling occurs under time/resource pressure?	Context notes for exception events

Source: Zajac (2026) and Guide to Health Informatics (2026a): interview + observation + process-tracing principles adapted to this routine study.

Note: Field notes were split into descriptive and reflexive notes; only descriptive notes were transcribed into the submitted CSV event log.

A.1 Core Deliverable Artefacts (Part I)

Table 2: Line-numbered self-interview transcript (conducted February 18, 2026)

Line	Transcript
L01	Interview question: Please describe your normal weekday morning routine from when you wake up until you leave home.
L02	Participant: My alarm goes off at 06:45, and I usually get up right away after stopping it.
L03	Participant: I first drink a glass of water, then I go to the bathroom.
L04	Participant: I normally take a quick shower, around ten to twelve minutes.
L05	Participant: After that I get dressed and prepare my bag for the day.
L06	Participant: I usually pack my laptop, charger, student ID card, wallet, and keys.
L07	Participant: Then I make coffee and toast.
L08	Participant: I normally sit down, eat breakfast, and check my calendar and transport timing on my phone.
L09	Participant: After breakfast I brush my teeth and do a final mirror check.
L10	Participant: I check the weather so I can choose the right jacket.
L11	Participant: Before leaving, I do a final essentials check: phone, wallet, keys, and card.
L12	Participant: I aim to leave at around 07:30 to catch transport to campus.
L13	Interview question: Which steps vary the most from day to day?
L14	Participant: Sometimes I snooze once if I am tired.
L15	Participant: Breakfast can change; if I am in a rush, I might just eat fruit instead of toast.
L16	Participant: Occasionally I spend a minute looking for keys if I did not put them in the same place.
L17	Interview question: Are other people typically involved in this routine?
L18	Participant: Mostly no, I do it alone, but once in a while I text my roommate if I need a shared item.
L19	Interview question: Which tools or systems are central in the routine?
L20	Participant: My phone alarm, calendar, weather app, the coffee machine, toaster, and my backpack.
L21	Interview question: How do you define start and end of this routine?
L22	Participant: It starts when the alarm sounds and ends when I close the apartment door behind me.

Table 3: Interview-based activity model (work-as-imagined / knowledge-as-stated)

ID	Activity label	Actor	Order	Expected time	Main tools/resources
A01	Stop alarm and orient to time	Participant	1	06:45-06:47	Smartphone alarm
A02	Drink water	Participant	2	06:47-06:50	Glass, tap
A03	Toilet and shower	Participant	3	06:50-07:02	Bathroom, shower
A04	Dress and pack bag	Participant	4	07:02-07:12	Wardrobe, backpack
A05	Prepare breakfast and coffee	Participant	5	07:12-07:20	Coffee machine, toaster
A06	Eat breakfast and review schedule	Participant	6	07:20-07:25	Phone calendar/transport apps
A07	Brush teeth and final grooming	Participant	7	07:25-07:28	Toothbrush, mirror
A08	Check weather and essentials	Participant	8	07:28-07:30	Weather app, keys, wallet, ID
A09	Leave apartment for commute	Participant	9	07:30	Front door, bag

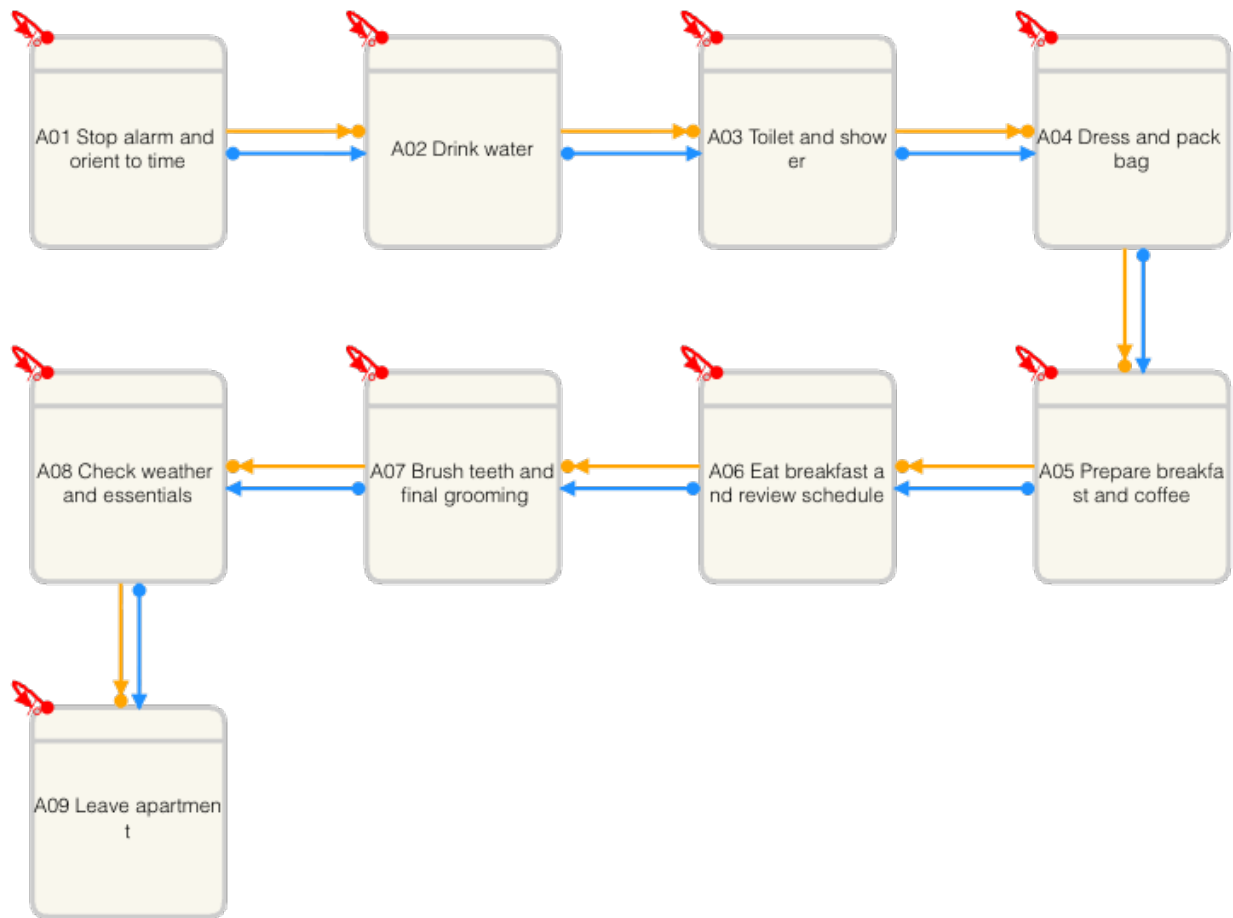


Figure 1: Interview-based DCR graph for the morning routine (A01-A09)

Source: Own DCR Graph export (February 2026), built from transcript evidence in Table 2 (lines L02-L12, L14-L18, L21-L22).

Note: Used as baseline work-as-imagined model for comparison with observed events in Deliverables 2 and 3.

Table 4: Observed event-log excerpt (February 20, 2026)

Event	Timestamp	Event label	Tool	Context note	Ref.
E01	2026-02-20 06:45	Alarm rings and snooze	Smartphone alarm	One snooze before wake-up.	A01
E03	2026-02-20 06:52	Reply to roommate message	Messaging app	Extra micro-coordination event.	NA
E05	2026-02-20 06:57	Toilet and shower	Bathroom	Hygiene block started later than interview expectation.	A03
E07	2026-02-20 07:12	Start coffee and toast	Coffee machine, toaster	Breakfast preparation begins.	A05
E08	2026-02-20 07:16	Switch breakfast to banana	Kitchen	Breakfast adapted due to no bread.	A05
E09	2026-02-20 07:18	Pack bag	Backpack	Packing occurs after breakfast prep starts.	A04
E10	2026-02-20 07:21	Find keys and borrow lock	Keys, messaging app	Workaround to solve missing item.	A08
E14	2026-02-20 07:30	Leave apartment	Front door	End of observed routine boundary.	A09

Table 5: Delta sheet between interview model and observed routine

Delta ID	Category	Interview ref.	Observed ref.	Difference description	Implication
D01	Missing events	A06	No direct event	No distinct seated breakfast-and-schedule-review event occurred.	Interview model overstates routine regularity.
D02	Extra events	None in baseline	E03 (06:52)	Short roommate-message coordination appears in observed routine only.	Reveals practical coordination load.
D03	Different order	A04 before A05	E07 then E09	Bag packing happened after breakfast preparation started.	Ordering constraints are weaker than stated.
D04	Timing distortions	A03 expected 06:50-07:02	E05 start 06:57	Hygiene block shifted later due to wake-up delays.	Time windows in interview are optimistic.
D05	Workarounds/coordination steps	A08 final check	E10 (07:21)	Missing lock triggered ad hoc borrowing via message.	Recovery actions are underrepresented in interview model.
D06	Relabeling/ambiguity	A05/A06	E08, E13	“Breakfast” changed from sit-down meal to banana plus coffee-to-go.	Label definitions need operational clarity.

B Supporting Tables and Code for Part II

This appendix contains full Part II evidence tables, visualisations, and the custom method code excerpt referenced from Deliverable 6 and Deliverable 7.

Table 6: Selected methods from Part 3

Method	Primary facets	RQ link	Why selected
M2	Data, Source, System	RQ1-3	Baseline completeness check before further interpretation.
M6	Data, Task, Human	RQ1, RQ3	Captures urgency and temporal representativity in one method.
M8 (15 min)	Task, System, Human	RQ2	Directly tests safety-critical communication conformance.
M10	Data, System, Task	RQ1	Combines TAT metrics with batch-logging sensitivity.
M11	Human, Task, System	RQ1, RQ3	Adds workload and actor-distribution context.

Table 7: Key results and interpretation

Method	Key result	Interpretation (incl. expectation check)	Main facet(s)
M2	High missingness in context attributes (e.g., <code>study_uid</code> 134/150).	As expected; process-flow analysis is possible, but context-dependent inference is limited.	Data, Source
M6	Urgency skew: 15 stat/urgent vs 2 routine; shift imbalance (Day 11, Night 6, Evening 0).	As expected; representativity is uneven, so generalisation should be cautious.	Data, Task, Human
M8	16 pass, 1 fail (critical case missing communication event).	Worked as intended; even one failed critical case is operationally important for RQ2.	Task, System, Human
M10	Batch logging suspected in 6/17; mean total TAT 147.93 min raw vs 198.50 min clean.	Most surprising output; RQ1 conclusions are highly sensitive to timestamp credibility.	Data, System, Task
M11	Events per role uneven (radiographer 64; resident 7; system actor high concentration).	As expected; workload concentration affects operational and modelling interpretation.	Human, Task, System

B.1 Notebook Visualisations (Part II)

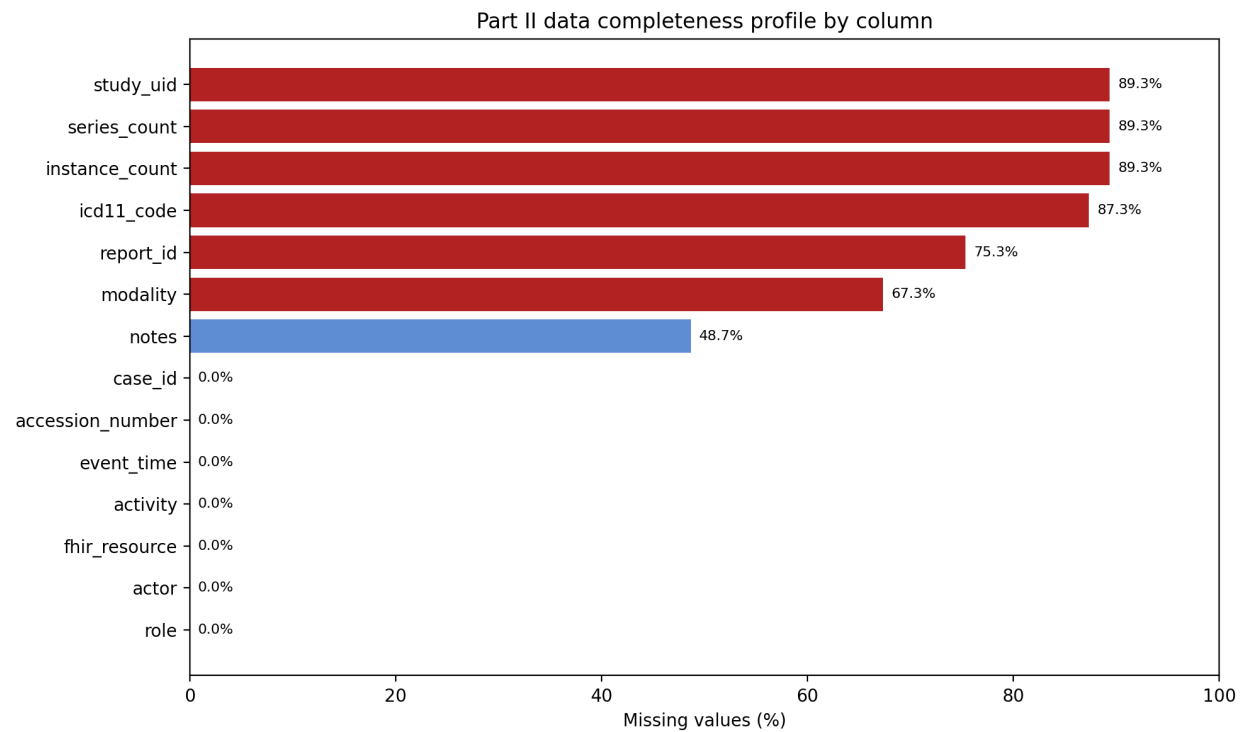


Figure 2: Part II visualisation: missingness profile by column (Method 2 context)
Source: Generated from *radiology_event_log.csv* using the Part II notebook analysis workflow (February 2026).

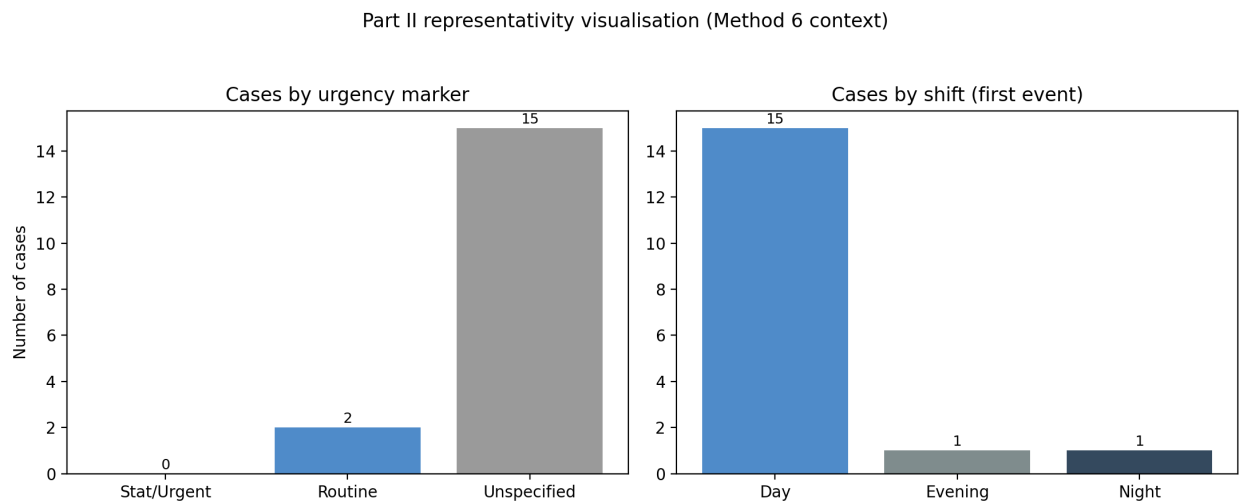


Figure 3: Part II visualisation: urgency and shift representativity (Method 6 context)
Source: Generated from *radiology_event_log.csv* using the Part II notebook analysis workflow (February 2026).

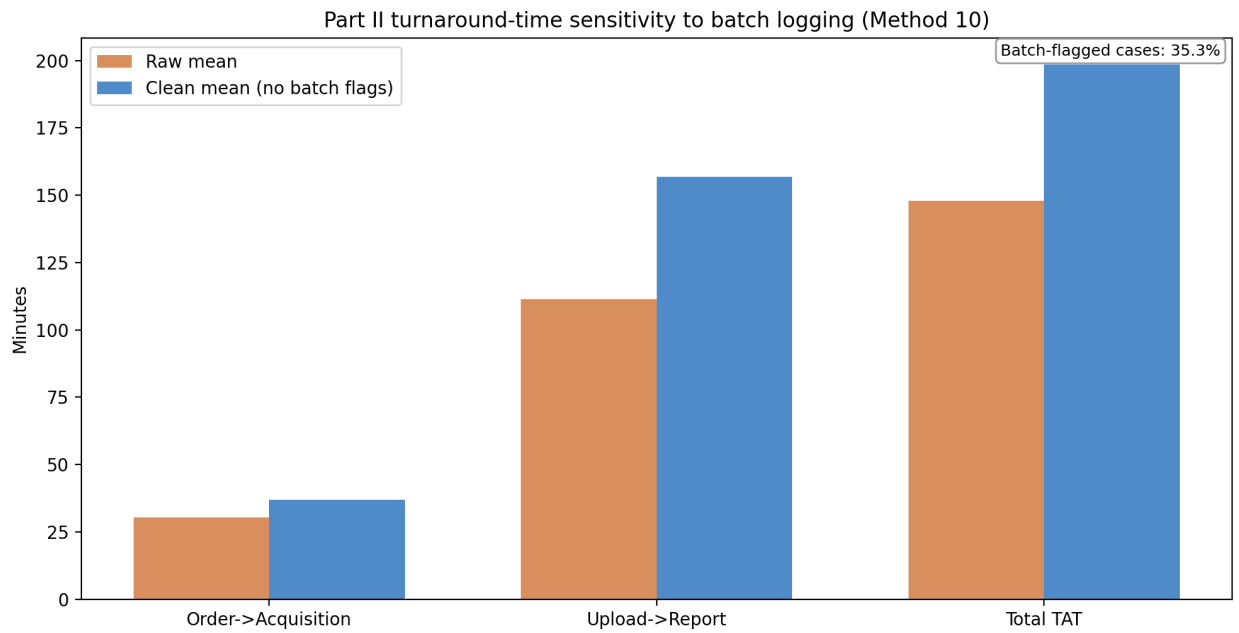


Figure 4: Part II visualisation: TAT sensitivity before and after batch-logging filtering (Method 10 context)

Source: Generated from *radiology_event_log.csv* using the Part II notebook analysis workflow (February 2026).

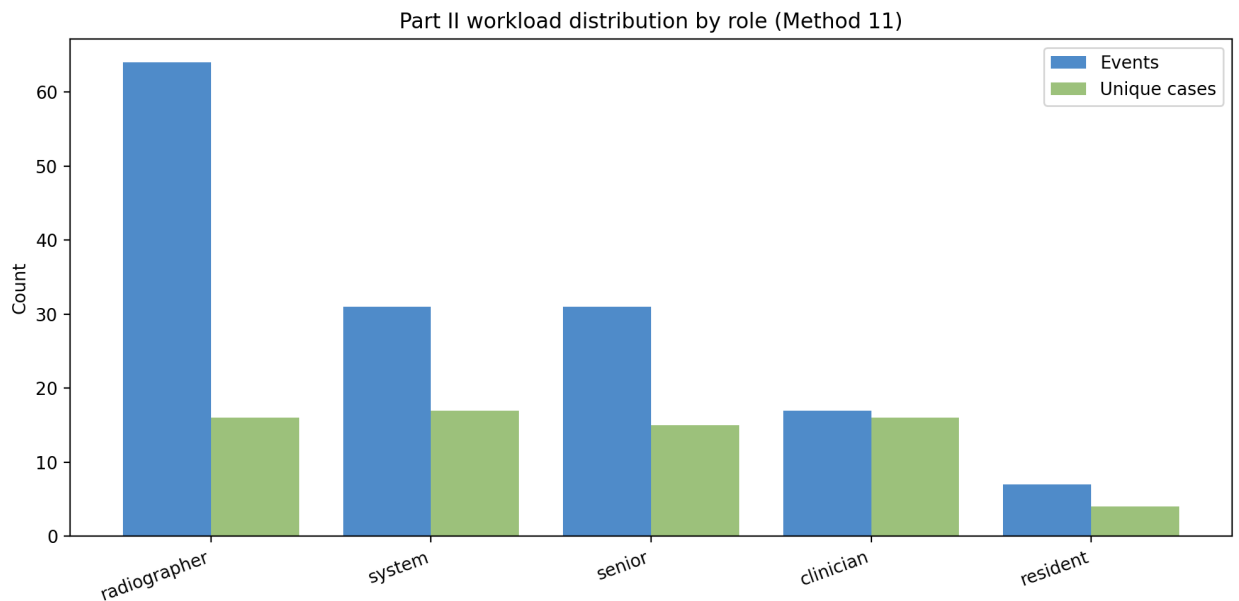


Figure 5: Part II visualisation: workload distribution by role (Method 11 context)

Source: Generated from *radiology_event_log.csv* using the Part II notebook analysis workflow (February 2026).

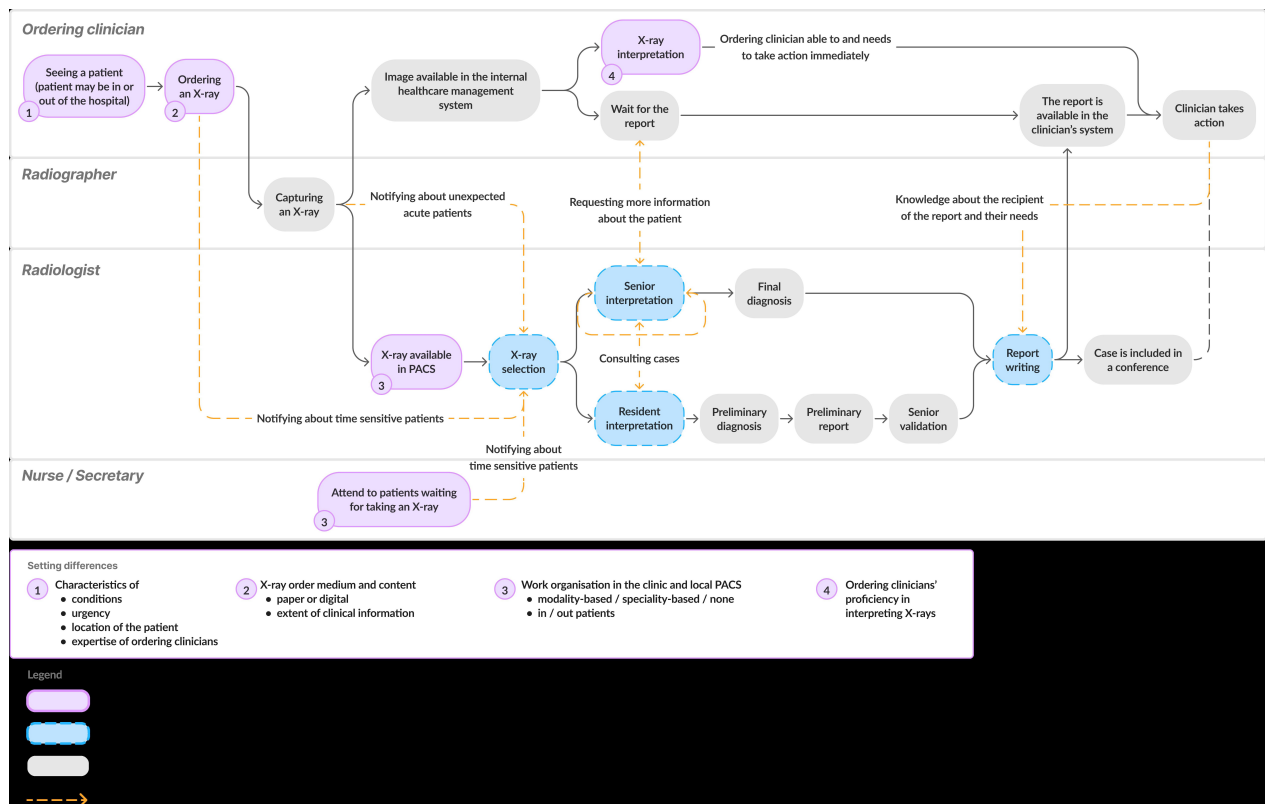


Figure 6: Assignment Figure 1: Observed radiology workflow

Source: Provided in Assignment 2 as *Figur1-assignment2.jpg*.

Table 8: Facet mapping for Batch Logging Credibility

Facet	Involvement	Justification
Data	++	Timestamp integrity is the direct object of assessment.
Source	++	Logging practice determines whether entries are real-time or delayed.
System	+	Interface/workflow design can incentivise retrospective entry.
Task	+	Timeliness-sensitive tasks are directly affected by timestamp quality.
Human	+	Workload pressure shapes documentation behavior.

B.2 Custom DQ Assessment Method Code Excerpt

```

1 def my_custom_dq_method(df: pd.DataFrame):
2     rows = []
3     for case_id, cdf in df.groupby('case_id'):
4         n = len(cdf)
5         u = cdf['event_time'].nunique()
6         dup = n - u
7         dur = ((cdf['event_time'].max() - cdf['event_time'].min()).total_seconds() / 60) if n > 1
            else 0

```

```

8     score = (2 if dup >= 3 else 1 if dup > 0 else 0) + (2 if n >= 5 and dur < 2 else 0) +
    (1 if (u/n) < 0.5 else 0)
9     risk = 'high' if score >= 4 else ('medium' if score >= 2 else 'low')
10    rows.append({'case_id': case_id, 'risk_level': risk, 'risk_score': score})
11    out = pd.DataFrame(rows)
12    flagged = out[out['risk_level'].isin(['high', 'medium'])]
13    return {'case_flags': out,
14            'summary_metrics': {'total_cases': len(out), 'flagged_case_rate_pct': round(100*
15                                len(flagged)/len(out), 2)},
            'examples': flagged.head(3).to_dict('records')}

```

Table 9: Invisible work not represented explicitly in the event log

Activity from diagram	Why not logged?	Facet	Consequence	Capture option
Radiologist-to-radiologist consultation	Verbal/ad hoc interaction	Human, Task	Understates collaborative interpretation work	Optional peer-consult event
Clarification with ordering clinician	Often outside core system	System, Source	Communication-delay attribution may be misleading	Linked communication event
Informal reprioritisation/handoff	Local coordination, not formal workflow step	Task, Human	Bottleneck and fairness analyses miss queue management	Priority-adjusted event with reason code

B.3 Textbook Figures Referenced in the Report

The figures below are included to support direct references in Part I and Part II. To separate them from assignment-produced figures, each caption explicitly preserves the textbook figure number.

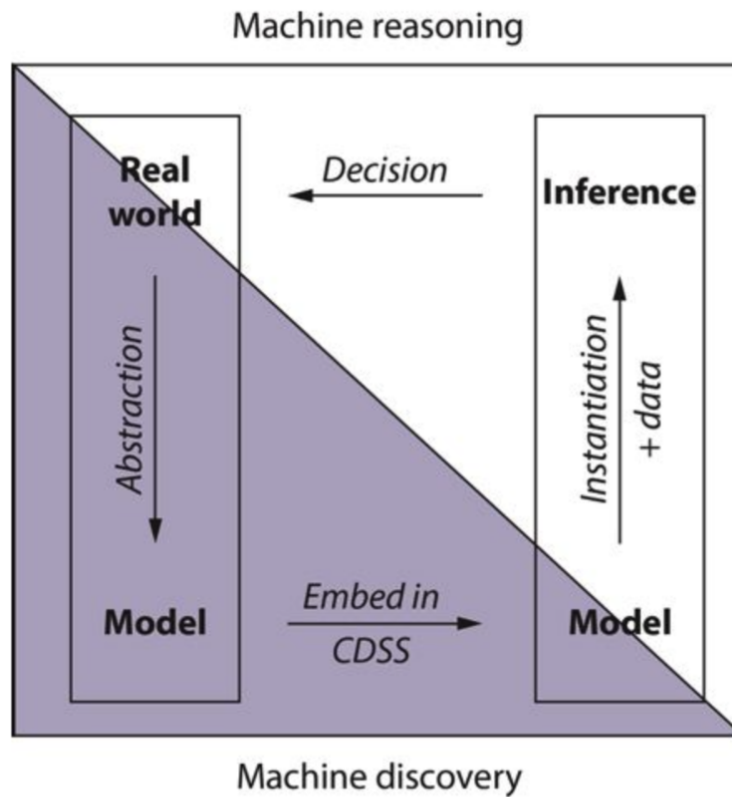


Figure 27.1 Computer reasoning depends first on acquiring models that record our understanding of how the world works and then applying those models in a decision support system to arrive at conclusions shaped by the data specific to a given decision task.

Figure 7: Textbook Figure 27.1 from *Guide to Health Informatics*: computer reasoning cycle from model construction through use

Source: *Guide to Health Informatics* (2026a). Reproduced from course compendium reading.

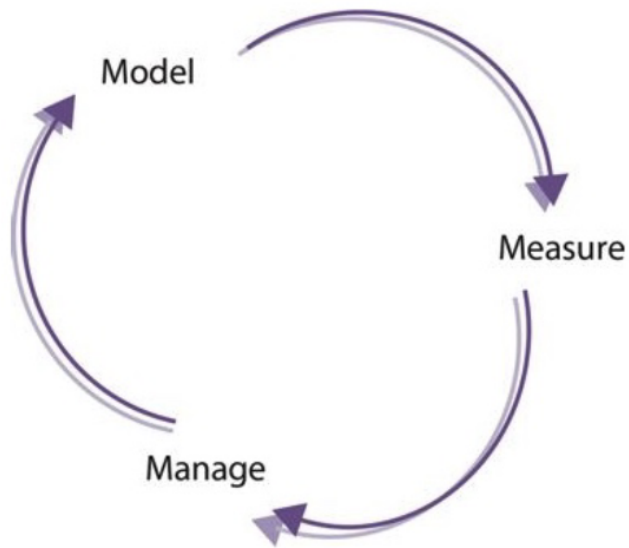


Figure 9.1 The model-measure-manage cycle. When the outcome of a management decision is fed back into another cycle, a feedback control loop is formed.

Figure 8: Textbook Figure 9.1 from *Guide to Health Informatics*: model-measure-manage cycle
Source: *Guide to Health Informatics* (2026b). Reproduced from course compendium reading.

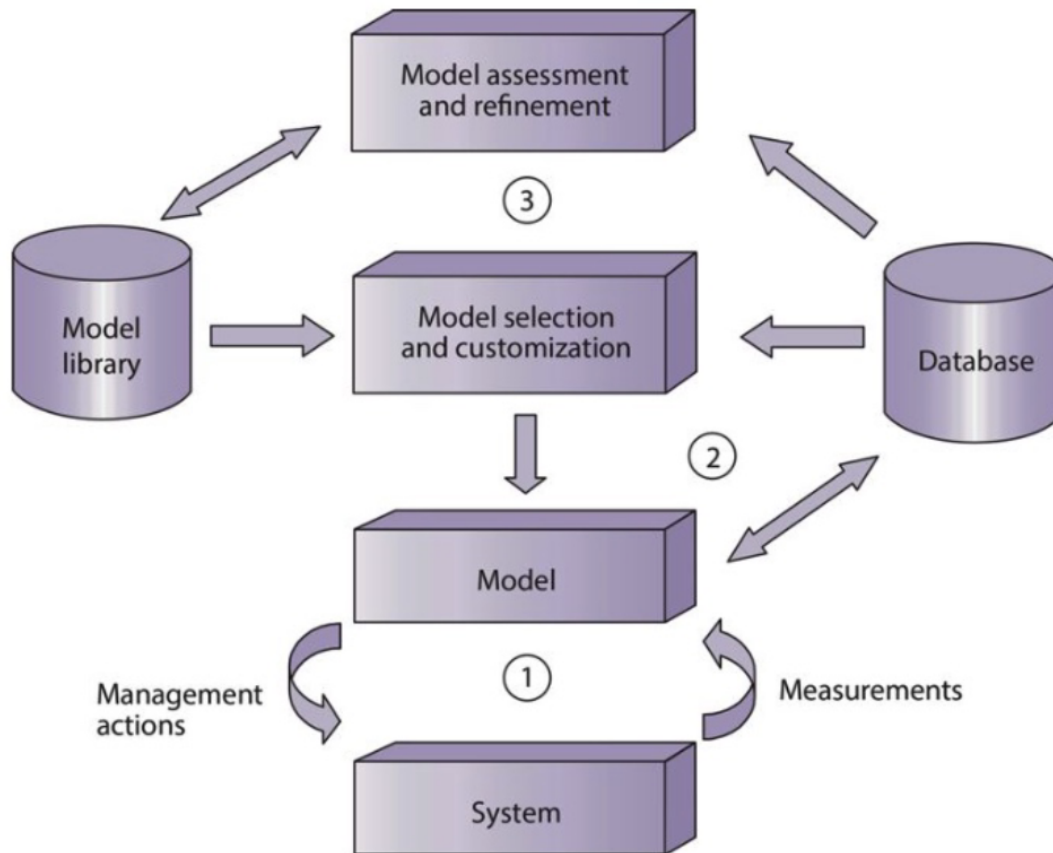


Figure 9.2 The three-loop model. Three separate information cycles interact within the health system. The first and second cycles select and apply models to the management of specific systems. The third information cycle tries to assess the effectiveness of these decisions and improve the models accordingly.

Figure 9: Textbook Figure 9.2 from *Guide to Health Informatics*: three-loop model
Source: *Guide to Health Informatics* (2026b). Reproduced from course compendium reading.

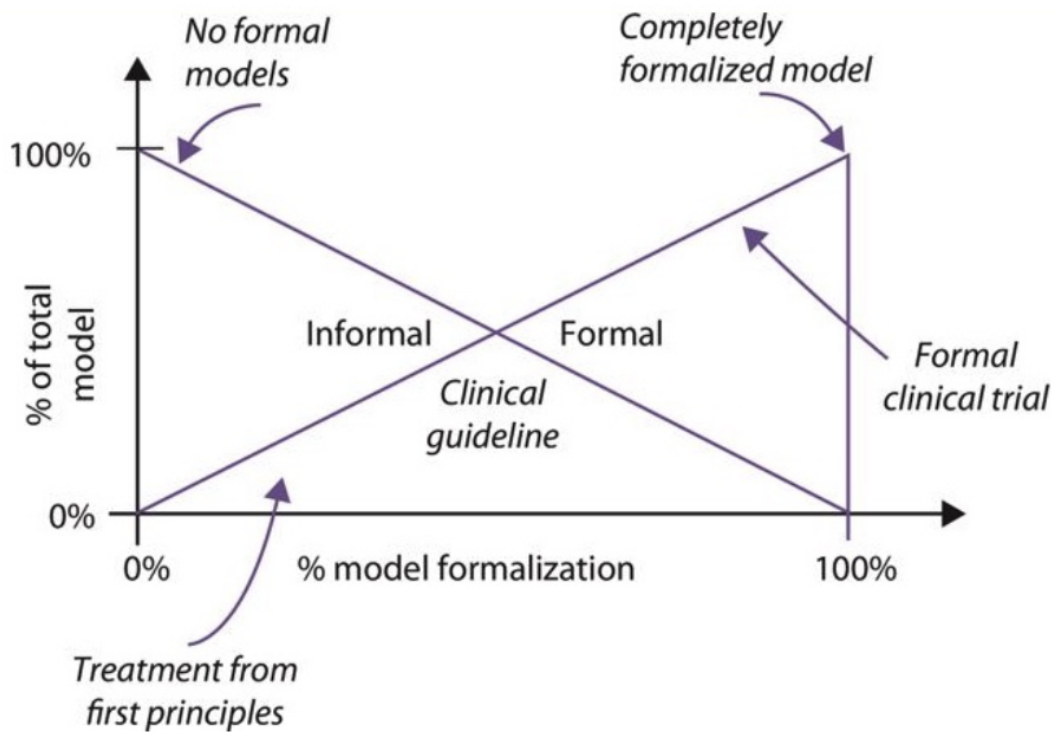


Figure 9.3 There is a continuum of possible formalization of information systems. Formal and informal models combine together to cover the total modelling needs of a given problem. The degree of formalization depends on the type of problem being addressed.

Figure 10: Textbook Figure 9.3 from *Guide to Health Informatics*: formalization continuum
 Source: *Guide to Health Informatics* (2026b). Reproduced from course compendium reading.

References

- Guide to Health Informatics (Course Compendium) (2026a). *Guide to Health Informatics: Selected Chapter 27*. PDF. Course reading PDF provided in NDAB20010U.
- (2026b). *Guide to Health Informatics: Selected Chapters 9 and 27*. PDF. Course reading PDF provided in NDAB20010U.
- Zajac, H. D. (Feb. 2026). *Sundhedsdata og interoperabilitet: Methods of knowing + Health-care data*. PDF. Course slides, NDAB20010U Health Data and Interoperability, University of Copenhagen.